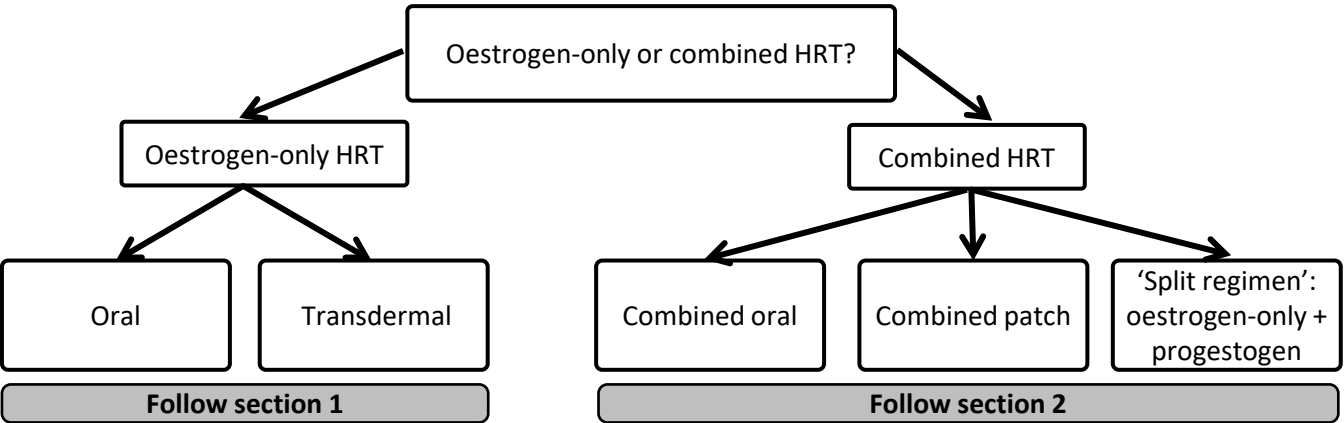


1. Troubleshooting HRT availability

The BMS Tools for Clinicians: Progestogens and endometrial protection, HRT preparations and equivalent alternatives; BMS guideline on management of unscheduled bleeding on HRT, 2024

- To use this GEMS:
- Identify what type of HRT a woman is taking and what component is unavailable.
 - Identify the dose equivalent of the unavailable component (section 1 for oestrogen-only, section 2 for combined).
 - Find a suitable available matching formulation that is acceptable to the woman (section 3).

What type of HRT is being used?



Women may experience some irregular bleeding following a switch to an alternative preparation. This should settle within 3–6 months. Seek medical advice if it doesn't.

Section 1: Oestrogen-only HRT

Oral oestrogen-only HRT options:

Make direct switch to another oral preparation of the equivalent dose (see HRT formulation tables). Consider switch to a transdermal preparation of an equivalent dose.

Transdermal oestrogen-only HRT options:

Swap to any transdermal oestrogen preparation of an equivalent dose. Patches may be cut to give a required dose (off-licence). If switching from transdermal to oral, do a full reassessment of suitability, i.e. avoid in higher-risk women (e.g. if >60y, overweight or migraine) due to increased VTE and CVA risk with oral preparations.

Dosing and dose equivalence (BMS guidance on management of unscheduled bleeding on HRT, 2024) Doses in *italics* are off-licence. There is no published evidence for the efficacy or safety of off-licence oestradiol.

Dosage	Oral oestradiol	Oestradiol patch	Oestradiol gel		Oestradiol spray
			Oestrogel	Sandrena	
Ultra-low	0.5mg	12.5mcg	1/2 pump	0.25mg	1 spray
Low	1mg	25mcg	1 pump	0.5mg	2 sprays
Standard	2mg	50mcg	2 pumps	1mg	3 sprays
Moderate	3mg	75mcg	3 pumps	1.5–2.0mg	4–5 sprays
High	4mg	100mcg	4 pumps	3mg	6 sprays

IMPORTANT! GMC guidance recommends that where a licensed preparation is available, it should be used in preference to an off-licence drug. If adjusting HRT for availability, we should take this into account.

We make every effort to ensure the information in these pages is accurate and correct at the date of publication, but it is of necessity of a brief and general nature. The information presented herein should not replace your own good clinical judgement, or be regarded as a substitute for taking professional advice in appropriate circumstances. In particular, we suggest you carefully consider the specific facts, circumstances and medical history of any patient, and recommendations of the relevant regulatory authorities. We also suggest that you check drug doses, potential side-effects and interactions with the British National Formulary. Save insofar as any such liability cannot be excluded at law, we do not accept any liability for loss of any type caused by reliance on the information in these pages. July 2025. For full references see the relevant Red Whale articles.

2. Troubleshooting HRT availability

The BMS Tools for Clinicians: Progestogens and endometrial protection, HRT preparations and equivalent alternatives; BMS guideline on management of unscheduled bleeding on HRT, 2024

Section 2: Combined HRT

Combined oral HRT options

- Switch to alternative oral preparation: match the progestogen if possible (*there are a few containing norethisterone*), but otherwise choose alternative with a different progestogen.
- Consider switching to a transdermal patch (e.g. Evorel Sequi has the same progestogen as Elleste Duet).
- Or offer a split regimen (e.g. Oestrogel + Provera contains the same hormones as Indivina).

Combined patch HRT options

- There are 2 combined patches, both of which have the same dose of oestrogen, so switching is possible:
 - Evorel (contains norethisterone) – applied twice weekly.
 - Femseven (contains levonorgestrel) – applied weekly.
- Or consider switching to oral equivalent of patch – this will need reassessment of risk factors.
 - Use with caution in women at higher risk of thromboembolism (e.g. if >60y, overweight or with migraine) due to increased VTE and CVA risk with oral preparations.
- Tibolone is a low-risk oral alternative to continuous combined HRT.
- Offer split regimen (see below).

'Split regimen' oestrogen-only HRT + progestogen options

Oestrogen component unavailable:

- See oestrogen-only alternatives (on page 1).

Progestogen component unavailable:

- Micronised progesterone often first line as considered safest option. If not available:
 - Alternatives are medroxyprogesterone acetate (MPA) or norethisterone (note: norethisterone is associated with VTE risk so avoid in higher-risk women) or
 - Offer IUS or
 - Offer combined product: patch or oral, *if appropriate* (e.g. Bijuve is a continuous combined oral HRT with micronised progesterone).

Dosing and dose equivalence (BMS guidance on management of unscheduled bleeding on HRT, 2024)

Doses in italics are off-licence. There is no published evidence for the efficacy or safety of off-licence progestogen.

Progestogen	Sequential dose	Continuous dose
Micronised progesterone (Utrogestan or Gepretix)	200mg daily on days 1–14 of month. <i>300mg daily with high-dose oestrogen off-licence.</i>	100mg daily from day 1–26 of cycle, or continuously. <i>200mg daily with high-dose oestrogen off-licence.</i>
	Note: oral preparation can be used vaginally off-licence if poorly tolerated orally.	
Medroxy-progesterone acetate	10mg daily on days 1–14 of month. <i>20mg daily with high-dose oestrogen off-licence.</i>	2.5mg daily with low-dose oestrogen. 5.0mg daily with standard/moderate-dose oestrogen. <i>10mg daily with high-dose oestrogen off-licence.</i>
Norethisterone	5mg daily on days 1–14 of month.	5mg daily. <i>1mg is adequate for endometrial protection, but no formulation is available so the BMS suggests we can use 3x 350mcg Noriday POPs daily off-licence.</i>
Any 52mg intrauterine system up to 5y (Mirena, Benilexa, Levosert), although only Mirena is licensed and for 4y.		

(Note: Evidence shows that women using sequential regimens with less than 10d of progestogen a month are at increased risk of endometrial hyperplasia. The BMS therefore recommends that a progestogen is given on 12–14d per month. For practicality, we suggest that the patient takes their progestogen from day 1 to 14 of the calendar month.)

3. HRT: formulations

(from MIMS 2025 and *The BMS Tools for Clinicians: Progestogens and endometrial protection HRT preparations and equivalent alternatives*)



Red Whale

GEMS
Guidelines & Evidence Made Simple

Oral: usually cheapest.

Transdermal: lower VTE/CVA risk so use preferentially if >60y, overweight or migraines.

Dosing: start with low-dose preparation (and match progestogen dose if using separate formulation – see *HRT regimens and progestogen doses* for details).

If urogenital symptoms predominate: consider using vaginal oestrogens instead.

Approximate prices per month:

£ = usually <£5

££ = usually £5–10

£££ = usually >£10

Combined HRT regimens

Sequential combined

Route	Oestrogen	Progestogen	Name	Cost (per month)
Oral	Oestradiol 1–2mg	Dydrogesterone 10mg (14 tabs)	Femoston	££
	Oestradiol valerate 2mg	Norethisterone 1mg (12 tabs)	Novofem	£
	Oestradiol 1mg		Elleste Duet	£
	Oestradiol 1–2mg			
	Oestradiol 1mg (10 tabs) 2mg (12 tabs)	Norethisterone 1mg (10 tabs)	Trisequens (phasic preparation)	£
Patch	Oestradiol 50mcg/24h	Norethisterone 170mcg per 24h (2 patches/w for 14d)	Evorel Sequi	£££

Continuous combined

Route	Oestrogen	Progestogen	Name	Cost (per month)
Oral	Conjugated equine oestrogen 300mcg	Medroxyprogesterone acetate 1.5mg	Premique Low Dose (also available generically)	£
	Oestradiol 0.5–1mg	Dydrogesterone 2.5–5.0mg	Femoston Conti	££
	Oestradiol 1–2mg	Medroxyprogesterone acetate 2.5–5.0mg	Indivina	££
	Oestradiol 1mg	Norethisterone 0.5mg	Kliovance	£
	Oestradiol 2mg	Norethisterone 1mg	Elleste Duet Conti	££
			Kliofem	£
	Oestradiol 1mg	Progesterone 100mg	Bijuve	££
Patch	Oestradiol 50mcg/24h	Norethisterone 170mcg per 24h (2 patches/w)	Evorel Conti	£££
	Oestradiol 50mcg/24h	Levonorgestrel 7mcg per 24h (1 patch/w)	Femseven Conti	£££

Other HRT preparations

Route	Content	Dose	Name	Cost (per month)
Oral	Tibolone (gonadomimetic)	2.5mg	Livial	££
			Generic tibolone	£

We make every effort to ensure the information in these pages is accurate and correct at the date of publication, but it is of necessity of a brief and general nature. The information presented herein should not replace your own good clinical judgement, or be regarded as a substitute for taking professional advice in appropriate circumstances. In particular, we suggest you carefully consider the specific facts, circumstances and medical history of any patient, and recommendations of the relevant regulatory authorities. We also suggest that you check drug doses, potential side-effects and interactions with the British National Formulary. Save insofar as any such liability cannot be excluded at law, we do not accept any liability for loss of any type caused by reliance on the information in these pages. July 2025. For full references see the relevant Red Whale articles.

4. HRT: formulations (continued)

(from MIMS 2025 and *The BMS Tools for Clinicians: Progestogens and endometrial protection; HRT preparations and equivalent alternatives*; BMS - management of unscheduled bleeding on hormone replacement therapy)



Red Whale

GEMS

Guidelines & Evidence Made Simple

Approximate prices per month:

£ = usually <£5

££ = usually £5–10

£££ = usually >£10

Oestrogen-only formulations and progestogens

Oestrogen-only (only if no uterus/Mirena in place)

Route	Oestrogen	Name	Cost (per month)
Oral	Conjugated equine oestrogen 300mcg/625mcg/1.25mg	Generic preparations	£
	Oestradiol 1–2mg	Elleste Solo	£
		Progynova	£
		Zumenon	£
Patch	Oestradiol 25/50/75/100mcg per 24h	Estraderm MX	££
		Evorel	£
	Oestradiol 25/37.5/50/75/100mcg per 24h	Estradot	££
	Oestradiol 50/75/100mcg per 24h	Femseven	££
	Oestradiol 50/100mcg per 24h	Progynova TS	££
Gel	Oestradiol gel (0.06%) 1–4 pumps (1.25–3.0mg) per 24h	Oestrigel 80g pump	£–££
	Oestradiol gel (0.1%) 0.5–1.5mg per 24h	Sandrena 0.5/1.0mg sachets	££
Spray	Oestradiol 1.53mg/metred dose 1–3 sprays per 24h	Lenzetto 8.1ml (56 actuations)	£–££

Progestogen/progesterone-only (to be used with oestrogen-only HRT if has uterus)

Route	Progestogen	Dose		Name	Cost
		Sequential (take daily for 12–14d of a cycle)	Continuous (take everyday)		
Oral	Medroxy-progesterone acetate	10mg with low/moderate-dose oestrogen 20mg with high-dose oestrogen (off-licence)	2.5mg with low-dose oestrogen 5.0mg with moderate-dose oestrogen 10mg with high-dose oestrogen (off-licence)	Provera is the only brand available, but is not licensed for HRT (although endorsed by BMS)	£–££
Oral	Norethisterone	5mg irrespective of oestrogen dose	1mg with low-dose oestrogen (this dose is not available so the POP Noriday (350mcg) may be used at a dose of 3 tablets a day as an off-licence alternative) 5mg for moderate to high-dose oestrogen	Generic norethisterone: not licensed for endometrial protection, but endorsed by BMS	££
Oral/ vaginal (vaginal use is off-licence)	Micronised progesterone	200mg with low/moderate-dose oestrogen 300mg with high-dose oestrogen (off-licence) (Note: licensed dose is 10–12d/cycle, but BMS recommends 12–14d)	100mg daily with low/moderate-dose oestrogen 200mg daily with high-dose oestrogen (off-licence) (Note: licensed dose is from d1–26 of a cycle, but BMS recommends everyday)	Utrogestan	££
				Gepretix	£
				Generic preparation	£
Intra-uterine	Levonorgestrel 52mcg	20mcg/24h		Mirena (licensed) Levosert or Benilexa (off-licence)	£

We make every effort to ensure the information in these pages is accurate and correct at the date of publication, but it is of necessity of a brief and general nature. The information presented herein should not replace your own good clinical judgement, or be regarded as a substitute for taking professional advice in appropriate circumstances. In particular, we suggest you carefully consider the specific facts, circumstances and medical history of any patient, and recommendations of the relevant regulatory authorities. We also suggest that you check drug doses, potential side-effects and interactions with the British National Formulary. Save insofar as any such liability cannot be excluded at law, we do not accept any liability for loss of any type caused by reliance on the information in these pages. July 2025. For full references see the relevant Red Whale articles.